Patient Name: Rafael Das

DOB: 11/27/1963

Reason for referral: iron deficiency

Patient phone: 530-555-2003

Primary insurance: Blue Cross

Dartmouth Health

Dartmouth Hitchcock Clinics

Phone: (603) 577-4081
Fax: (603) 676-4080
Medically Urgent Fax: (603) 640-1909

Patient Name: Rafael Das

Referring Provider: _____ Office Phone: _____

DOB: 11/27/1963

Practice Name: _____ Fax: _____

Reason for referral: iron deficiency

Practice Address _____ PCP Name: _____

Patient phone: 530-555-2003

Primary insurance: Blue Cross

Patient Name: _____ MRN# _____

DOB: _____ Cell Phone _____ Home Phone _____ Work Phone _____

Mailing Address: _____

Will a supplied interpreter be needed for this appointment? ❑ No ❑ Yes Language: _____

Health Insurance: _____ Subscribers Name: _____

Policy #: _____ Group# _____ Subscribers DOB _____

Referral for Direct Endoscopy / Colonoscopy

❑ **Office Consultation** – FAX ALL office notes, reports, labs, etc. to (603) 577-4081

Diagnosis: _____

Testing done: _____

❑ **EGD** ❑ **Bravo**

❑ Abdominal Pain ❑Abnormal Radiographic Testing ❑ Celiac Disease Confirmation ❑ Dyspepsia

❑ Dysphagia ❑ Gastric Ulcer F/U ❑ GERD ❑ GI Bleed ❑ Iron Deficiency ❑ Barrett's Esophagus

❑ Other: _____

❑ **Diagnostic Colonoscopy**

❑ Abnormal Radiographic ❑ Testing ❑ Chronic Diarrhea ❑ Chronic Constipation ❑ FU Diverticulitis

❑ GI Bleed ❑ Hemocult Positive Stool ❑ Iron Deficiency ❑ Personal HX Colon Cancer

❑ Personal HX Colon Polyps: ❑ Adenomatous ❑ Vilous Adenoma ❑ Other: _____

Screening Colonoscopy

❑ Age 50 or above ❑ FM Hx Colon CA-1st degree relative or multiple 2nd degree relatives

❑ Previous screening colonoscopy: Year _____ ❑ Other: _____

Please answer Yes or No to ALL questions:

❑ Yes ❑ No Cardiac Disease

❑ Yes ❑ No CABGCAD

❑ Yes ❑ No MI /CVA

❑ Yes ❑ No Pulmonary Disease

❑ Yes ❑ No Latex Allergy

❑ Yes ❑ No Atrial Fibrillation

❑ Yes ❑ No Heart Failure

❑ Yes ❑ No Diabetes - If Yes: Insulin Dependent: ❑ Yes ❑ No

❑ Yes ❑ No Renal/Kidney Disease ❑ Yes ❑ No

❑ Yes ❑ No Hemophilia/other blood disorders:
(if yes please explain): _____

Medications

Taking ASPIRIN OR NSAIDS? ❑ Yes ❑ No Stop 5 days prior to procedure ❑ Yes ❑ No

Taking Coumadin or Plavix,? ❑ Yes ❑ No Stop 5 days prior to procedure? ❑ Yes ❑ No

Patient will resume Coumadin/Plavix one day after procedure unless otherwise instructed

Rate your patient's ASA classification: _____ (3 or 4 needs anesthesia support)

1) Healthy patient with no disease outside of the surgical process.
2) Systemic disease that does NOT alter active daily living.
3) Systemic disease that DOES alter active daily living.
4) Severe incapacitating disease process that is a threat to life.

ClinLab
Clinical Testing Laboratory, Inc.

Clinical Testing Lab of California
55492 Spring Street, Suite 201
Vienna, CA 94078-9932
Phone: 510-555-1212

Specimen Number	Patient ID	Control Number	Account Number	Account Phone Number	Route

Patient Last Name	Account Address

Patient First Name	Patient Middle Name
Jeff	

Patient SS#	Patient Phone	Total Volume

Age (Y/M/D)	Date of Birth	Sex	Fasting
46 v.o.		Male	Yes

Patient Address	Additional Information

Date and Time Collected	Date Entered	Date and Time Reported	Physician Name	NPI	Physician ID

Tests Ordered
CBC, Platelet Ct, and Diff ; Liver Function (AST/ALT) ; Iron/Iron Binding Panel

General Comments

TESTS	RESULT	FLAG	UNITS	REFERENCE INTERVAL	LAB
CBC, Platelet Ct, and Diff					
Hematocrit	40.6		%	40.8 - 51.9	07
Hemoglobin	14.2		g/dL	14.6 - 17.8	07
Red Blood Cell Count	8.4		x10E6/uL	4.69 - 6.07	07
White Blood Cell Count	7.4		x10E3/uL	3.2 - 10.6	07
RDW	13.0		%	10.8 - 14.1	07
MCV	82.6		fL	77.8 - 94.0	07
MCH	27.4		pg	26.5 - 32.6	07
MCHC	34.4		g/dL	32.7 - 36.9	07
Imm.Granulocytes (Absolute)	6.2		x10E3uL	1.3 - 7.0	07
Granulocytes (Percent)	57.2		%	44.0 - 76.0	07
Eosinophils (Absolute)	0.3		x10E3uL	0.0 - 0.4	07
Eosinophils (Percent)	5.2		%	0.0 - 6.0	07
Basophil (Absolute)	0.08		x10E3uL	0.0 - 0.1	07
Basophil (Percent)	1.7		%	0 - 2	07
Monocytes (Absolute)	0.64		x10E3uL	0.2 - 0.7	07
Monocytes (Percent)	6.9		%	4 - 8.9	07
Lymphocytes (Absolute)	2.26		x10E3uL	0.8 - 3.1	07
Lymphocytes (Percent)	37.2		%	15 - 43	07
Platelets (Absolute)	388		x10E3uL	177 - 406	07
Liver Function					
Aspartate aminotransferase (AST)	223	High	U/L	8 - 43	07
Alanine Aminotransferase (ALT)	257	High	U/L	7 - 45	07
Iron and Total Iron Binding					
Serum Iron	2300	Very High	ug/dL	40 - 155	07
Total Iron Binding Capacity	328		ug/dL	250 - 400	07
Transferrin Saturation	72	Very High	%	20 - 50	07

PRELIMINARY INTERPRETATION:

A preliminary diagnosis of Hemochromatosis can be made based on very high Serum Iron and Transferrin Saturation values. Elevated liver enzyme levels are indicative of liver failure which is a common consequence of Hemochromatosis.

There are well established pathogenic genetic variants that are strongly correlated with Hereditary hemochromatosis. The presence of a hereditary disorder has consequences for family members. Genetic testing and counseling should be considered by the patient and family members.